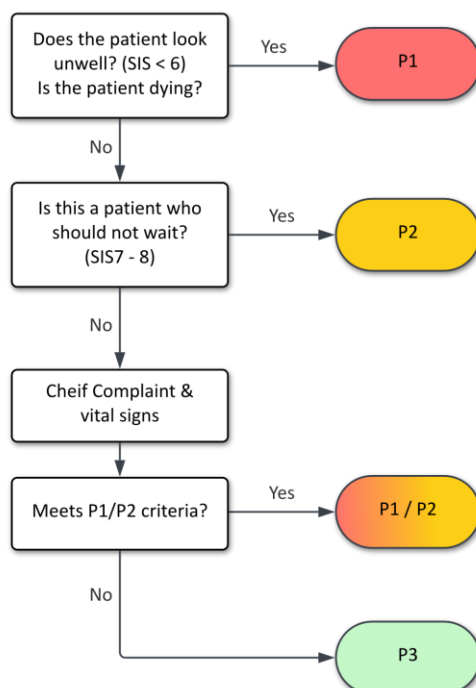


PEDIATRIC PATIENT ACUITY CATEGORY SCALE (PACS)

PACS	Definition	Time Seen	Clinical State	Clinical Diagnosis / Presentation
P1	Resus	STAT	- SIS ≤ 6 - Severe Respiratory Distress (any of the following) - SpO₂ < 92% - RR > 20/min over upper limit, or > 5/min below lower limit - Grunting, deep retractions, head bobbing - Cyanosis - Signs of hypoperfusion (any of the following) - Hypotension - CRT > 2s, cold peripheries - Thready pulses - Lethargy (poor interaction with environment/people, poor eye contact) - Altered mental state / GCS < 14. - Metabolic derangement with end-organ dysfunction - Altered mental status, seizures. - Hemodynamic instability, cardiac arrhythmias	- Cardiac arrest. - Hypertensive emergency (AMS, headaches, seizures, blurring of vision) - Anaphylaxis or severe allergic reactions with cardiorespiratory instability or impending respiratory compromise. - On-going seizures. - Major trauma (any of the following): - High risk mechanism* - Hemodynamic instability - Associated with fractures with neurovascular compromise. - Severe injuries - Depressed skull fracture, open head injury. - Fracture of ≥ 2 long bones, pelvis. - Penetrating injuries to head, neck, chest, abdomen, groin. - Traumatic amputation. - Severe maxillofacial injury. - Suspected spinal cord injury. - Major burns: full thickness, >10% TBSA, or involving face. - Poisoning - With substances of high toxicity - With altered mental state or cardiorespiratory instability
P2	Non-Resus	15 mins	- SIS 7 – 8 - Mild – Moderate Respiratory Distress. (Any of the following) - SpO₂ < 95% - RR > 10/min over upper limit - Intercostal, suprasternal retractions, nasal flaring - Pain score ≥ 6, regardless of etiology - Fever - Rectal temperature $\geq 38^{\circ}\text{C}$ (age < 3 months) - Hyperpyrexia $\geq 41^{\circ}\text{C}$ (any age) - High risk patients with fever and/or symptoms of decompensation (lethargy, vomiting) - Inborn Errors of Metabolism (IEM) - Immunosuppressed patients (oncology patients, transplant patients, patients on immunosuppressants) - Transfer from other restructured hospitals / urgent care clinics - Age ≤ 5 years with injuries that require repair/procedure. - All infants < 3 months.	- Post-ictal with return of consciousness. - Burns / scalds > 5 to 10% TBSA. - Ocular trauma (sharp/blunt/chemical), or severe eye pain. - Allergic reactions without respiratory compromise (rash or periorbital edema). - Stable poisoning. - Button battery in ear, nose, or ingested. - Testicular pain. - Psychiatric presentations - Post-suicide attempt - Acute psychosis - Violent/aggressive patient - Referrals from private clinics for (appropriate) escalation of higher acuity community cases: - Abnormal vital signs (except temperature) - Cases that require surgical review (e.g., suspected appendicitis, testicular pain, abscess, fractures etc.) - Metabolic derangement with no/mild symptoms - Lethargy, GCS 14 – 15 - No ECG changes or hemodynamic instability
P2	Non-Resus	Admin	Patients that require intensive monitoring & resources to reflect manhour invested, e.g., procedural sedation, fracture reduction, Bier block, IV fluids etc.	Administrative PAC to reflect CE resources invested into such cases.
P3	Emergency	60 mins	SIS 9 – 10	Acute complaints (≤ 2 weeks)

*HR mechanism: Fall from 10m (or > 3 body lengths), RTA with shattered windscreen, bent steering wheel, ejection of patient, intrusion into patient's compartment, prolonged extraction time, death of any occupant, vehicle roll over with unrestrained patient, motor vehicle vs pedestrian > 32km/h

1. CONCEPTUAL ALGORITHM FOR TRIAGE



- Patients should be assigned the higher priority acuity if more than 1 PACS criteria is met.
- Consultation PACS should reflect the highest PACS status achieved while in CE

2. MODIFIED SEVERITY INDEX SCORE (SIS)

Add points	0	1	2	
Subtract from 10	(-2)	(-1)		
Activity Level	AV <u>P</u> U	AV <u>V</u> PU	AV <u>P</u> U	P1 = SIS ≤ 6 P2 = SIS 7 – 8 P3 = SIS 9 – 10
Behavior	Lethargic (interacting poorly with environment)	Irritable	Normal, Playing	
Color	Mottled & grey; CRT ≥ 5	Pale; CRT 3 – 4s	Pink; CRT ≤ 2s	
Respiratory effort	Moderate to deep intercostal, suprasternal retractions, head bobbing, SCM use, grunting. ≥5 L/min O ₂	Mild to moderate intercostal, suprasternal retractions, nasal flaring. <5 L/min of O ₂	No respiratory distress No tachypnea, distress, or O ₂	
Temperature	< 36.3°C or ≥ 41°C	38.3°C to 40.9°C	36.3°C to 38.2°C	

- Calculate SIS by subtracting from a total of 10 points:
 - 2 to -3 points → P2
 - 4 or more points → P1

3. NORMAL VITAL SIGNS FOR AGE (PEDIATRIC PATIENTS)

Age	Heart Rate		Respiratory Rate		Min. Blood Pressure	
	Min.	Max.	Min.	Max.	SBP	DBP
Newborn to < 1 month	90	180	30	60	< 60 mmHg	< 2/3 of SBP
1 to < 3 months	90	160	30	60	< 70 mmHg	
3 to < 6 months	80	160	30	60		
6 to < 12 months	80	140	25	45		
1 to < 6 years	75	130	20	30	< 70 + (age in years x 2)	
6 to < 10 years	70	110	16	24		
10 to < 15 years	60	90	14	20	< 90 mmHg	< 60 mmHg
≥ 15 years	60	90	12	16		

4. USEFUL REFERENCES

4.1 WEIGHT ESTIMATION

Age	Formula
0 – 12 months	Weight (kg) = (age in months x 0.5) + 4
1 – 5 years	Weight (kg) = (age in years x 2) + 8
6 – 12 years	Weight (kg) = (age in years x 3) + 7

4.2 SIGNS OF RESPIRATORY DISTRESS

	Mild	Moderate	Severe
Airway	Stridor on exertion/crying	Stridor at rest	Stridor at rest
Behavior & feeding	Normal Full sentences	Some irritability Difficulty talking/crying Difficulty feeding	Increased irritability and/or lethargy Unable to talk/cry Unable to feed
Respiratory Rate	Normal to slight increase	> 10 /min over	> 20 /min over
Accessory muscles	Mild intercostal Suprasternal retractions	Moderate intercostal & suprasternal retractions Nasal flaring	Severe intercostal, suprasternal, & sternal retractions
Oxygen requirements	No O ₂ requirements	<5 L/min	≥5 L/min